INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Ceritinib monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00340
Version 4

> Once daily with food; no rest week

Treatment stages

01 **Continuous oral ceritinib**
Daily with food

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Ceritinib blocks abnormal ALK signals that help some lung cancers grow. In advanced cancer after crizotinib, the aim is to control cancer and delay progression. The choice depends on your tumour test, previous treatment, other options and priorities.

ALK TARGETED THERAPY

Ceritinib

Blocks abnormal ALK growth signalling

ALK-positive tumour confirmed → Growth signal blocked → Cancer growth slowed

Ceritinib inhibits ALK signalling in a tumour with a confirmed ALK rearrangement. It can affect healthy tissues too, so blood tests and symptom checks continue even when treatment is taken at home.

Report breathing changes, fainting, jaundice, severe abdominal pain, persistent diarrhoea/vomiting or marked thirst promptly.

Your treatment and its purpose

Confirm the setting

Adults with ALK-positive advanced non-small-cell lung cancer previously treated with crizotinib: NCCP 00340a. This suite does not turn the broader first-line licence into an NCCP indication or funding approval. Confirm your treatment setting with the team.

NCCP 00340 v4 · official regimen

Duration and review

The usual starting dose is 450 mg once daily with food, every day without a planned rest week. Follow your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

NCCP 00340 v4 · official regimen · EMA Zykadia product information, retrieved 29 October 2026

Evidence after crizotinib and for dosing with food

Study results describe groups, not an individual forecast.

Trials used different doses and populations. No proven individual benefit or cross-trial ranking is implied.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Contact oncology urgently for new or worsening cough or breathlessness, fainting, palpitations, severe dizziness, yellow skin or eyes, dark urine, severe persistent abdominal pain, persistent vomiting or diarrhoea, poor drinking or little urine, or marked thirst with frequent urination and feeling unwell. Severe breathing difficulty, collapse or severe chest pain needs emergency help.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

Ceritinib checks over time

Columns are milestones, not equal durations. The team books actual tests and reviews; no patient-entered dates are needed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00340 v4 · official regimen · EMA Zykadia product information, retrieved 29 October 2026

THE WHOLE TREATMENT JOURNEY

Ceritinib monotherapy: treatment and checks

Your treatment pattern

Once daily with food; no rest week

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ALK-positive advanced non-small-cell lung cancer (NSCLC) · after crizotinib

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First 3 months	After month 3	Benefit review	Hold / change / stop
Daily ceritinib NCCP 00340 · SMPC	Confirm ALK, previous crizotinib and prescription	Daily with food; no planned rest week	Continue authorised daily schedule	Assess benefit and tolerability	Written instructions for any change
Liver tests NCCP 00340 · SMPC	alanine aminotransferase (ALT), aspartate aminotransferase (AST) and bilirubin	Every 2 weeks	Monthly; sooner if abnormal	Continue tests and symptom review	Check recovery before any restart
Blood count, kidneys and glucose NCCP 00340 · SMPC	full blood count (FBC), renal profile and fasting glucose	Every 4 weeks; glucose sooner if needed	Every 4 weeks and when indicated	Review diabetes, symptoms and medicines	Follow unresolved abnormalities
Heart and pancreatic checks NCCP 00340 · SMPC	electrocardiogram (ECG), pulse/blood pressure (BP), lipase and amylase	ECG after 1 month; other checks as needed	ECG/ electrolytes and pancreatic enzymes as indicated	Review pulse/ BP and symptoms	Investigate fainting or abdominal pain

Care / check	Before starting	First 3 months	After month 3	Benefit review	Hold / change / stop
Symptoms, interactions and supply NCCP 00340 · SMPC	Review medicines, food and contact plan	Report breathing/ gastrointestinal (GI) symptoms promptly	Check adherence, prescriptions and supply	Scans and disease assessment per consultant	Confirm follow-up and who authorises restart

What each check is for

1 Safe start

Before first dose

Correct biomarker, treatment line and safety context matter.

Ask / act: Confirm dose, food instructions, support medicines and contacts.

2 Early liver surveillance

Every 2 weeks for 3 months

Liver injury may occur without early symptoms.

Ask / act: Confirm test bookings and result reviewer.

3 Regular clinical checks

Throughout daily treatment

Continuous tablets still need continuing review.

Ask / act: Review symptoms, interactions, intake, supply and scans.

4 Dose held or treatment stopped

Toxicity, progression or changed priorities

Restart criteria differ by toxicity and food schedule.

Ask / act: Prescriber gives a new written plan and follow-up.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Daily treatment, tests and next decisions

No personal dates are needed. Confirm actual appointments with your team.

Treatment or check	Timing in this view
Preparation	**Before starting** Confirm tumour test, prior treatment, baseline checks and medicines
Daily capsules	**First 3 months → After month 3 → Benefit review** Take the prescribed dose with food every day
Blood tests and reviews	**First 3 months → After month 3 → Benefit review → Hold / change / stop** Fortnightly early liver tests; regular blood, glucose and heart checks
Next plan	**Hold / change / stop** Clinician authorises continuation, hold, change or stop

Do not alternate fed and fasting dosing.

Columns are milestones, not equal time periods.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Taking ceritinib at home

Food changes absorption, so the dose and food instructions belong together.

Daily, missed or vomited doses

Take at the same time daily with food. Swallow capsules whole with water; do not chew or crush. Take a missed dose unless the next is due within 12 hours; ask your pharmacist if timing is unclear. Never double a dose. If you vomit, do not replace the dose; take the next at its usual time.

If you cannot eat

If illness prevents you eating with the dose, contact the team. Only the prescriber can arrange a different fasting dose. Do not alternate between fed and fasting schedules or change the dose yourself.

Tests and reviews

Baseline tests and ongoing checks do different jobs.

Your monitoring plan

Before starting: ALK test, blood count, kidney and liver tests, pancreatic enzymes, fasting glucose, ECG and symptom/medicine review. Blood count, kidneys and fasting glucose every 4 weeks; liver tests every 2 weeks for the first 3 months, then monthly. ECG after 1 month, then as needed. Extra heart, electrolyte, glucose or pancreatic tests may be needed for risks, symptoms or abnormal results; scans follow the consultant plan.

Symptoms can bring a review forward

Report new symptoms between visits. Tests alone cannot establish whether symptoms are caused by treatment.

Support during treatment

Ask for help with symptoms, daily routines and practical concerns.

Prevent and treat nausea

Ceritinib has moderate-to-high oral sickness risk. Ask for daily prevention and rescue instructions. Your team must check heart-rhythm and medicine interactions when choosing anti-sickness treatment; do not add medicines or a steroid course yourself.

NCCP 00340 v4 · official regimen · NCCP systemic anticancer therapy (SACT) Antiemetic Guidance V6 (2025), Tables 5 and 8 · NCCP supportive antiemetics for NCIS Medical Oncology; check local policy

Check interactions

Avoid grapefruit or grapefruit juice and St John’s wort. Check prescription, pharmacy and herbal medicines before use, including heart medicines, antibiotics, antifungals, seizure treatments and acid-reducing medicines. Do not stop a prescribed medicine without advice.

Pregnancy, fertility and feeding

Discuss pregnancy, fertility and breastfeeding before treatment. Women who could become pregnant need highly effective contraception during treatment and for 3 months afterwards; hormonal contraception may be less effective, so agree a suitable method. Do not breastfeed during this treatment. Fertility effects are uncertain; ask about preservation if relevant.

Diarrhoea, fluids and daily life

Use the agreed diarrhoea treatment promptly. Report persistent symptoms, poor intake or dehydration. Ask for help with appetite, fatigue and practical support; avoid driving or machinery if fatigue or visual symptoms affect safety.

Symptoms needing prompt help

Keep the oncology number available day and night.

Do not wait for the next visit

Contact oncology urgently for new or worsening cough or breathlessness, fainting, palpitations, severe dizziness, yellow skin or eyes, dark urine, severe persistent abdominal pain, persistent vomiting or diarrhoea, poor drinking or little urine, or marked thirst with frequent urination and feeling unwell. Severe breathing difficulty, collapse or severe chest pain needs emergency help.

When you call

Tell the team the medicine, last dose and new symptoms; do not delay calling to collect details.

Benefit and choices

A study after crizotinib and platinum chemotherapy found longer disease control with ceritinib than the chemotherapy used at that time. It tested an older fasting dose. A separate dose study supports taking the current lower dose with food; its main efficacy group had not had previous treatment. These studies cannot predict your outcome or rank all current treatment options.

Match evidence to your situation

ASCEND-5 involved previous crizotinib and platinum chemotherapy and an older fasting dose. ASCEND-8 supports the fed schedule but its main efficacy analysis was treatment-naive. Ask how these results fit your previous treatments and current alternatives.

ASCEND-5: post-chemotherapy/crizotinib phase III trial, Shaw 2017 · ASCEND-8: pharmacokinetic and food-effect study, Cho 2017 · ASCEND-8: primary efficacy and safety analysis, Cho 2019



Review your choices

The team reviews symptoms, scans, side effects and priorities. Discuss alternatives, supportive care and choosing not to proceed; you may reconsider consent.

Practical support

Prevent and treat nausea

Food and daily doses

Check interactions

Pregnancy, fertility and feeding

Diarrhoea, fluids and daily life

Taking tablets / attending treatment

Practical treatment details

Refills and interruptions

Request repeats early.

Questions to ask

Do I need to enter dates?

No. Your team books actual appointments; this map shows general milestones.

Explain the plan back: Can you explain taking the prescribed dose with food, the 12-hour missed-dose instruction, what to do if you cannot eat, your next liver test and when to call urgently?

Advanced ALK-positive non-small-cell lung cancer (NSCLC) after crizotinib Adults with ALK-positive advanced non-small-cell lung cancer previously treated with crizotinib: NCCP 00340a. This suite does not turn the broader first-line licence into an NCCP indication or funding approval. Confirm your treatment setting with the team.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Ceritinib daily oral treatment

Active medicines: ceritinib

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Individually selected daily oral antiemetic prophylaxis	Prescriber/pharmacy to select and prescribe under local oral-systemic anticancer therapy (SACT) policy	Daily while required during oral treatment; review prevention and rescue needs NCCP V6 Table 5 supports daily oral 5-HT3 antagonist prophylaxis for moderate-high risk. Ceritinib can prolong QT and cause bradycardia; check electrocardiogram (ECG)/electrolytes and interactions before selecting an agent. Do not copy an infusion steroid/NK1 bundle.

Additional phase-specific support

- Ceritinib has moderate-to-high oral sickness risk. Ask for daily prevention and rescue instructions. Your team must check heart-rhythm and medicine interactions when choosing anti-sickness treatment; do not add medicines or a steroid course yourself.
- If illness prevents you eating with the dose, contact the team. Only the prescriber can arrange a different fasting dose. Do not alternate between fed and fasting schedules or change the dose yourself.
- Avoid grapefruit or grapefruit juice and St John's wort. Check prescription, pharmacy and herbal medicines before use, including heart medicines, antibiotics, antifungals, seizure treatments and acid-reducing medicines. Do not stop a prescribed medicine without advice.
- Discuss pregnancy, fertility and breastfeeding before treatment. Women who could become pregnant need highly effective contraception during treatment and for 3 months afterwards; hormonal contraception may be less effective, so agree a suitable method. Do not breastfeed during this treatment. Fertility effects are uncertain; ask about preservation if relevant.
- Use the agreed diarrhoea treatment promptly. Report persistent symptoms, poor intake or dehydration. Ask for help with appetite, fatigue and practical support; avoid driving or machinery if fatigue or visual symptoms affect safety.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Advanced ALK-positive non-small-cell lung cancer (NSCLC) after crizotinib Adults with ALK-positive advanced non-small-cell lung cancer previously treated with crizotinib: National Cancer Control Programme (NCCP) 00340a. This suite does not turn the broader first-line licence into an NCCP indication or funding approval. Confirm your treatment setting with the team.

NCCP 00340 · VERSION 4

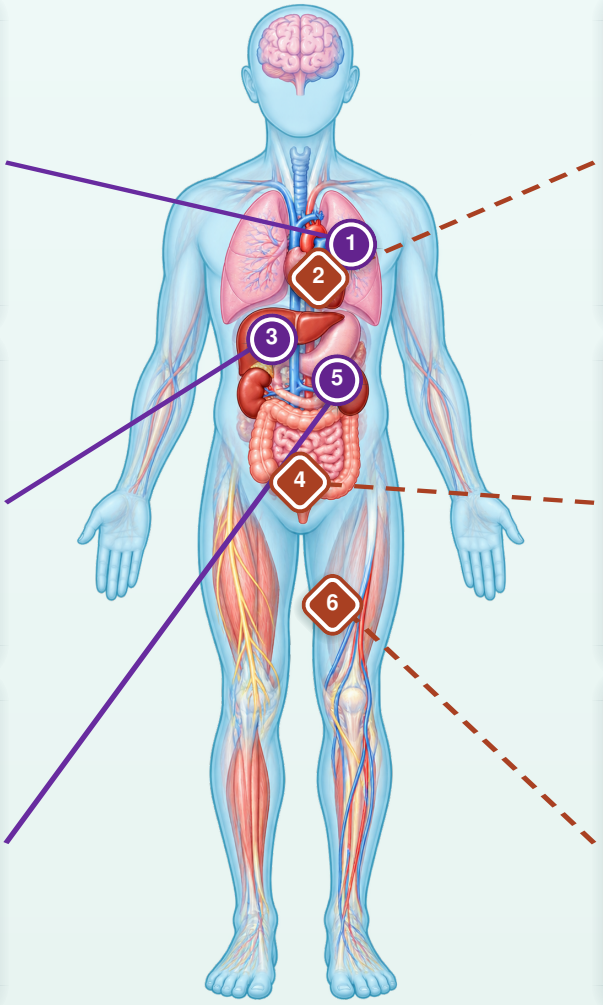
Ceritinib monotherapy

The usual starting dose is 450 mg once daily with food, every day without a planned rest week. Follow your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Oral targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 New cough or breathlessness

ceritinib

Possible lung inflammation needs urgent assessment.

2 Fainting or palpitations

ceritinib

Slow pulse and rhythm changes can be serious.

3 Yellow skin or dark urine

ceritinib

Blood tests detect liver changes; symptoms need urgent advice.

4 Diarrhoea or vomiting

ceritinib

Persistent symptoms can cause dehydration.

5 Persistent abdominal pain

ceritinib

Pancreatic inflammation needs prompt assessment.

6 Thirst and frequent urination

ceritinib

High glucose may need urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology urgently for new or worsening cough or breathlessness, fainting, palpitations, severe dizziness, yellow skin or eyes, dark urine, severe persistent abdominal pain, persistent vomiting or diarrhoea, poor drinking or little urine, or marked thirst with frequent urination and feeling unwell. Severe breathing difficulty, collapse or severe chest pain needs emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Nausea	☐	☐	☐	
Vomiting	☐	☐	☐	
Diarrhoea	☐	☐	☐	
New cough or breathlessness	☐	☐	☐	
Palpitations or fainting	☐	☐	☐	
Abdominal pain	☐	☐	☐	
Thirst / frequent urination	☐	☐	☐	
Fatigue	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Contact oncology urgently for new or worsening cough or breathlessness, fainting, palpitations, severe dizziness, yellow skin or eyes, dark urine, severe persistent abdominal pain, persistent vomiting or diarrhoea, poor drinking or little urine, or marked thirst with frequent urination and feeling unwell. Severe breathing difficulty, collapse or severe chest pain needs emergency help. Emergency symptoms: 112 / 999.

Carry your treatment details

Advanced ALK-positive NSCLC after crizotinib Adults with ALK-positive advanced non-small-cell lung cancer previously treated with crizotinib: NCCP 00340a. This suite does not turn the broader first-line licence into an NCCP indication or funding approval. Confirm your treatment setting with the team.

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Ceritinib monotherapy

NCCP 00340 v4

The usual starting dose is 450 mg once daily with food, every day without a planned rest week. Follow your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Contact oncology urgently for new or worsening cough or breathlessness, fainting, palpitations, severe dizziness, yellow skin or eyes, dark urine, severe persistent abdominal pain, persistent vomiting or diarrhoea, poor drinking or little urine, or marked thirst with frequent urination and feeling unwell. Severe breathing difficulty, collapse or severe chest pain needs emergency help.

https://sactcheck.com/patient/00340/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.25 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00340 v4 · pNCCP 00340 v4 p5; classification Tables 5/8; current SmPC interactions**
   https://healthservice.hse.ie/documents/6917/340_Ceritinib_monotherapy.pdf
2. **EMA Zykadia product information, retrieved 29 October 2026**
   https://www.ema.europa.eu/en/documents/product-information/zykadia-epar-product-information_en.pdf
3. **NCCP systemic anticancer therapy (SACT) Antiemetic Guidance V6 (2025), Tables 5 and 8**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf
4. **NCCP supportive antiemetics for NCIS Medical Oncology; check local policy**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **ASCEND-5: post-chemotherapy/crizotinib phase III trial, Shaw 2017**
   https://pubmed.ncbi.nlm.nih.gov/28602779/
6. **ASCEND-8: pharmacokinetic and food-effect study, Cho 2017**
   https://pubmed.ncbi.nlm.nih.gov/28729021/
7. **ASCEND-8: primary efficacy and safety analysis, Cho 2019**
   https://pubmed.ncbi.nlm.nih.gov/30851442/